

COLLAPSE/FALLS

Give urgent attention to the patient who has collapsed and any of:

- Collapse following vaccination →13
- New sudden asymmetric weakness or numbness of face, arm or leg; difficulty speaking or visual disturbance: consider **stroke** or **TIA** →136
- Decreased consciousness →16
- Fit →19
- Chest pain →37
- Difficulty breathing →38
- Glucose < 3 (or < 4 if diabetes) ↗ 17
- Sudden collapse and any of: generalised itch/rash, face/tongue swelling, wheeze, difficulty breathing, abdominal pain, vomiting or exposure to possible allergen¹, check for **anaphylaxis** ↗ 20
- Recent injury
- Systolic BP < 90
- Pulse < 50 or irregular
- Palpitations
- Family history of collapse or sudden death
- Abnormal ECG
- Known heart problem
- Collapse with exercise
- Vomited blood or blood in stool
- Pregnant or missed/overdue period with abdominal pain and vaginal bleeding
- Severe back or abdominal pain

Manage and refer urgently:

- If BP < 90/60, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.

Approach to the patient who has collapsed not needing urgent attention:

- Ensure patient has had an ECG. If abnormal, refer same day.
- Check Hb: if <12 (woman) or < 13 (man), **anaemia** likely ↗ 27.
- Screen for alcohol/drug use. In the past year, has patient: 1) drunk ≥ 4 drinks²/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
- Check BP: if ≥ 140/90 ↗ 132. Then measure BP after lying for 5 minutes and repeat after standing for 3 minutes. Does systolic BP drop by ≥ 20 or diastolic BP drop by ≥ 10?

Yes

Orthostatic hypotension likely

- This is common in the elderly.
- Review medications: e.g. fluoxetine, amitriptyline, amlodipine, enalapril, furosemide, hydrochlorothiazide, isosorbide dinitrate can cause syncope. Discuss with doctor.
- If diarrhoea ↗ 46, if vomiting ↗ 45, if fever ↗ 24, if poor fluid intake, encourage fluids and give **oral rehydration solution**.
- Advise patient to sit first before standing up from lying down.
- Refer if:
 - Diabetes
 - Peripheral neuropathy (pain/numbness of feet)
 - Tremor, slow movements or stiffness
 - History of constipation or erection problems

No

Was patient breathing very quickly or deeply immediately before or during the collapse?

No

Did patient have dizziness, light-headedness, nausea, sweating, weakness or vision changes before the collapse?

Yes

Common faint likely

- Advise to avoid triggers like overheating, dehydration and prolonged standing.
- Advise to lie flat with legs raised as soon as symptoms occur.

No

- If collapse associated with coughing, swallowing, head turning, refer.
- If known diabetes ↗ 130.

Yes

Hyperventilation likely

- Reassure and encourage patient to breathe at a normal rate.
- Assess for stress and anxiety ↗ 86.

- If none of the above, look for and manage likely cause: if vision problems ↗ 31, joint problems ↗ 62, foot problems ↗ 66, leg problems ↗ 65, dementia ↗ 148.
- Refer if patient > 65 years with possible heart disease, patient collapses/falls repeatedly or cause for collapse/falls is uncertain.

¹Common allergens include medication, food or insect bite/sting within the past few hours. ²One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer.